



جوستاف روسي انترناشيونال - مصر (EPE) * طلب علاج أول مرة / متردد

إسم المريض ربيعاً : احمد حسن عابد حسن
المحافظة : الجيزة
الحى / المدينة : البدرشين
رقم البطاقة : 27709232100738 رقم التليفون : 01235468526
العنوان : مزغونة
العمر (تاريخ الميلاد) : 23-09-1977 المهنة : ربه منزل
تاريخ تسجيل الإستمارة : 02-10-2026

الإقرار

أقر انا الموقع أدناه انني لا اتمتع باي نظام علاج مثل:- التأمين الصحي-القوات المسلحة-الشرطة-الهيئات-المؤسسات او اي نظام يتعارض مع نظام العلاج علي نفقة الدولة- وسوف اتحمل كافة نفقات العلاج في حالة ثبوت غير ذلك
الإخصائي الاجتماعي
التوقيع: (-----)
المقرر بما فيه
التوقيع: (-----)

التشخيص الإكلينيكي : سرطان بالثدي (مبادرة)

نتائج الأبحاث : (مرفق)

التشخيص النهائي : سرطان بالثدي (مبادرة)

خطة العلاج : علاج مبادرة سرطان الثدي علاج تدعيمي وتلطيفي [6579] قسم العلاج : الاورام

التعليق على التشخيص : -----

تقرير اللجنة الثلاثية المتخصصة

الاسم: د/محمد كيلاني
الاسم: د/ندى حسن
الاسم: د/نرمين رمضان
الوظيفة: -----
الوظيفة: -----
الوظيفة: -----
التوقيع: -----
التوقيع: -----
التوقيع: -----
أطلعت اللجنة علي لائحة العلاج الخاصة بالمجالس وتلتزم جهة العلاج بلائحة المجالس الطبية. كما تقرر اللجنة بان البيانات المدونة في هذا النموذج صحيحة.
تعتمد صحة البيانات المرفقة
رئيس اللجنة ومدير
المستشفى.

تعهد المستشفى أن لا يتم تنفيذ القرار إلا بعد مراجعة الملف الطبي للمريض وإعادة مناظرة المريض بالمستشفى قبل الصرف.

أعضاء اللجنة

د. / ----- د. / ----- د. / ----- د. / -----

قرار اللجنة

التشخيص النهائي : -----
خطة العلاج : -----
المستشفى : -----
جهة العلاج : -----
ملاحظات : -----
قسم : -----
نفقات : -----

ختم
شعار الجمهورية

مدير عام
مديرية الشؤون الصحية

رئيس
المجلس الطبي المتخصص

--

Name: احمد حسن عابد حسن National ID: 27709232100738	
--	---

تقرير طبي

السيد
تحية طبية وبعد,,

إيماء للطلب المقدم من	بشأن تقرير طبي للمريض
بقسم:	رقم طبي:

نتشرف بان نوضح فيما يلي تقريرنا عن حالته
--

تاريخ وساعة الدخول : -- --:-- yyyy-mm-dd	تاريخ وساعة الخروج : PM 01:17 05-07-2026
---	---

وبعد الكشف والفحوصات والمتابعة وجد انه
--

المریضة تعاني من ورم غير حميد بالثدى و تحتاج الي Supportive and palliative Treatment
--

التاريخ : 05-07-2026	الوقت : -- --:--
	الطبيب المعالج : د/شيماء لاشين



 جمهورية مصر العربية
 وزارة الداخلية
 بطاقة هوية شخصية

الاسم: حسن علي حسن
 مركز الإقامة: مركز النور - الجيزة
 رقم الهوية: ٢٧٧٠٩٢٣٢١٠٧٣٨

LF1933147



 ٢٠٢٥/٠٦ ٢٧٧٠٩٢٣٢١٠٧٣٨
 بون على

النوع: ذكر
 تاريخ الميلاد: ٢٠٢٢/٠٧/١٧
 المنطقة: محافظة الجيزة



Patient Name	: أحمد محمد علاء الدين / أحمد	Request No.	: 13000123
Patient No.	: 27709232100738	Request Date	: 31/07/2025
Path No.	: SR-1055/25	Verification Date	: 23/08/2025
Sex	: Male	Financial Category	: جروب روسي - فئة الدولة
Age	: 47 Y	Department	: Out Patient
Referring Doc :		Registered by	: marwa mohamed youss

Pathology Report

Diagnosis

Right axillary lymphadenopathy, slide&block revision ;
Nodular lymphocytic predominant Hodgkin lymphoma.

Signature
Prof. Dr. Gamal Abdelatty
Dr. Menan Hosam .

Verified by :
Dr. Hiba Hasan

Printing Date : 02/09/2025 17:10:09

Page 2 of 2

Printing By : marwa mohamed youss

Room No. (4) - First Floor-Building No. 287- Nile Corniche - Mar El Qadima - Cairo, Egypt

Name: احمد حسن عابد MR. : 1037	
Detailed Medical Report	
السيد	
تحية طيبة وبعد,,	
إيماء للطلب المقدم من	بشأن تقرير طبي للمريض
بقسم:	رقم طبي:
ننشرف بأن نوضح فيما يلي تقريراً عن حالته	
تاريخ وساعة الدخول :	تاريخ وساعة الخروج :
وبعد الكشف والفحوصات والمتابعة وجد انه ACASE NODULAR sclerosis Hodgkin lymphoma received 3cycles CHOP Interim PET CT regressive coarse THEN 3 CYCLES CHOP Last PET CT pending. A case of NLPHL s/p 6 cycles of CHOP, with intial PR after 3 cycles then PD after 3 more. For ICE X 4 then new PET then ASCT. To start cycel 1 ICE after labs. Progressed as stage 3b disease. In view of shortage of holoxan to start first cycle GDP clinical pharmacy note : wt 95 ht 175 sa 2 for d1 and d8 gemzar 2000 mg cisplatin 75 mg For C1D8 gemzar 1gm/m2 after labs with oral formulated mouth wash. Marked improvment of the general condition for C2 ICE with oral augmentin & mouth wash for pharyngitis which improved markedly PET CT 11/7/2026 total resolution of supra and infradiaphragmatic LN apart from few metabolically active rt axillary Ln 2cm suv 6.5 was 4.1*2.7cm suv 17 . Patient referred for palliative radiotherapy didn't go yet mean while for C4 ICE pharmacy visit: for ICE C5 + mabthera 500mg today	
تحريراً في ,	
التاريخ :	19-09-2026
الطبيب المعالج :	هاجر حمدي عبدالهادي محمود
يعتمد ,	

SUVmax~14, while the most active is seen at retro-caval group measuring 2x1.8 cm with SUVmax~16.9.

- No FDG avid lung nodules or masses detected.
- Bilateral pulmonary scattered atelectatic bands.
- No metabolically active pericardial or pleural lesions detected.
- Physiologic FDG uptake is noted in the heart.

Abdomen and Pelvis:

- Average sized spleen with increased heterogenous FDG uptake of SUVmax~7.2 at its lower pole, likely infiltrated.
- Few low grade FDG avid porta-hepatic lymph nodes reaching 1.7 cm with SUVmax~3.3
- Physiologic diffuse FDG uptake is seen over average sized liver. No FDG avid focal lesions detected
- Physiologic diffuse FDG uptake is seen over average sized spleen with homogenous parenchymal texture and no FDG avid focal lesions seen.
- Normal appearance of both kidneys with no stones or back-pressure changes. Physiologic FDG excretion is seen in the kidneys and bladder.
- Normal appearance of the pancreas and both supra-renal with no FDG avid lesions detected.

• No evidence of ascites or abdominal collections noted.

Musculoskeletal:

- Diffusely activated bone marrow with low grade FDG uptake, likely reactive for lab correlation.
- Few small non FDG avid left femoral and pelvic bones sclerotic osseous lesions, likely bone islands.
- No focal active blastic or lytic bone lesions detected.

Conclusion:

FDG PET/CT study for a known case of HL revealed;

- Multiple FDG avid supra-diaphragmatic lymphomatous nodal lesions.
- Few low grade metabolically active upper abdominal lymph nodes for close follow up.
- Average sized spleen with increased heterogenous FDG uptake as described, likely lymphomatous infiltration.

Reported By,

Dr. Mohamed Abdeltawab, MSc

Dr. Mohamed Mohsen, MD

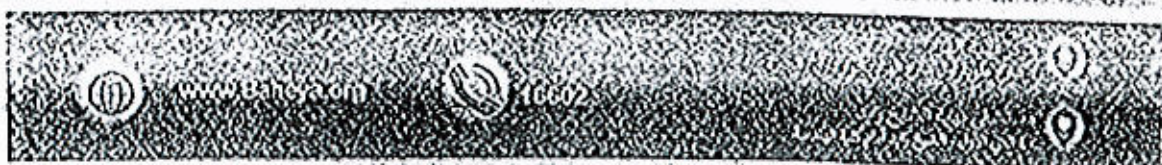
Ass. Prof. Mai Amr, Nuclear Medicine Consultant

30-Jul-2025 10:22 AM

PATIENT: AHMED HASSAN ABO HASSAN, 10125267832, Male, 47 y

Report version 3, 30-Jul-2025 10:22 AM

Page 2 of 2



جمهورية مصر العربية
بطاقة تحقيق الشخصية

احمد
حسن عابد حسن
مزغونه
مركز البدرشين - الجيزة

١٩٧٧/٠١ ٢٧٧.٠٩٢٣٢١.٠٠٧٣٨

LF1933147

٢٢
٢٥
٢٥



٢٠٢٥/٠١ ٢٧٧.٠٩٢٣٢١.٠٠٧٣٨

بدون عمل
متزوج مسلم ذكر

البطاقة سارية حتى ٢٠٢٢/٠٦/١٧

١

أحمد هارح
١٠٩٨١٢٠٧٨٠
الطبخ

فتح ملف محمد المرام ٢٠٢٥

٢٠١٩
٢٠٢٥

د. محمد عبد
٢٠٢٥/٩

٢٧

اسم المريض ثلاثي:
 رقم الملف: ٥١٩/٢٥

Doctor's Orders اوامر الطبيب

التشخيص: NCLPHL نوع التغذية:

توقيع الطبيب	الأوامر	الوقت	التاريخ
4/9/2025	PT. CT. Stage III B. Cervical, SCEN Axillary, mediastinal, Abdominal & sple pathology :- NCLPHL CD20+ve, CD3+ve, CD5+ve A&T2, Ki67 20% CD30 & CD15 → -ve ECHO :- 5.9 virology :- HIV 1 & 2 -ve HBsAg -ve HCV A.B For CHOP cycles then reassess full lts		

جميع الأوامر الطبية لابد أن تكون واضحة وموقعة من الطبيب

PATIENT NAME: AHMED HASSAN ABED HASSAN**GENDER:** Male**DATE :** 29-Jul-2025 11:35 AM**PATIENT ID:** 10125267832**AGE:** 47 y**ORDER:** 3492571**Whole body [18F] FDG PET-CT Scan****CLINICAL HISTORY:**

-47 years old male patient, presented with recently diagnosed HL.
Reason for the study: detection of FDG avid lymphomatous lesions.

TECHNIQUE:

- **Radiopharmaceutical:** 18F FDG, intravenously.
- **Time from 18F-FDG injection to scan:** 60 min
- **PET/CT images** were acquired from the skull through the upper thighs & CT images were acquired for the purpose of anatomical localization and attenuation correction. Images were reviewed in the transaxial, coronal, and sagittal planes.
- **Patient's weight:** Kg - **Patient's height:** cm - **BI. Gluc. Level:** mg/dL
- **Additional notes:** SUV max normalized to body weight was used.
- **Reference hepatic activity (SUV max):** 2.7
- **Reference mediastinal blood pool activity (SUV max):** 2

FINDINGS:**Head and Neck:**

- **Multiple FDG avid right lower cervical and right supra-clavicular lymph nodes (levels III, IV and Vb) reaching 1.8x2.2 cm with SUVmax~13.8 at the latter group.**
- Normal diffuse brain activity with no hypermetabolic brain lesions.
- Physiologic FDG uptake is seen in the nasopharynx, oropharynx, salivary glands and larynx.
- Physiologic FDG uptake is seen in submandibular and parotid salivary glands.
- Normal appearance of thyroid gland with no FDG avid nodules detected.

Chest:

- **Multiple FDG avid right axillary lymph nodes (all levels), reaching 4.5x3.3 cm with SUVmax~14.6**
- **Multiple FDG avid mediastinal right para-tracheal, retro-caval, pre-vascular, posterior mediastinal, carinal and sub-carinal lymph nodes, the largest is seen at the sub-carinal group measuring 3.3x2.3 cm with**

PATIENT: AHMED HASSAN ABED HASSAN, 10125267832, Male, 47 y

Report version 3, 30-Jul-2025 10:22 AM

Page 1 of 2



SUVmax~14, while the most active is seen at retro-caval group measuring 2x1.8 cm with SUVmax~16.9.

- No FDG avid lung nodules or masses detected.
- Bilateral pulmonary scattered atelectatic bands.
- No metabolically active pericardial or pleural lesions detected.
- Physiologic FDG uptake is noted in the heart.

Abdomen and Pelvis:

- **Average sized spleen with increased heterogenous FDG uptake of SUVmax~7.2 at its lower pole, likely infiltrated.**
- **Few low grade FDG avid porta-hepatic lymph nodes reaching 1.7 cm with SUVmax~3.3**
- Physiologic diffuse FDG uptake is seen over average sized liver. No FDG avid focal lesions detected
- Physiologic diffuse FDG uptake is seen over average sized spleen with homogenous parenchymal texture and no FDG avid focal lesions seen.
- Normal appearance of both kidneys with no stones or back-pressure changes. Physiologic FDG excretion is seen in the kidneys and bladder.
- Normal appearance of the pancreas and both supra-renal with no FDG avid lesions detected.
- No evidence of ascites or abdominal collections noted.

Musculoskeletal:

- Diffusely activated bone marrow with low grade FDG uptake, likely reactive for lab correlation.
- Few small non FDG avid left femoral and pelvic bones sclerotic osseous lesions, likely bone islands.
- No focal active blastic or lytic bone lesions detected.

Conclusion:

FDG PET/CT study for a known case of HL revealed;

- **Multiple FDG avid supra-diaphragmatic lymphomatous nodal lesions.**
- **Few low grade metabolically active upper abdominal lymph nodes for close follow up.**
- **Average sized spleen with increased heterogenous FDG uptake as described, likely lymphomatous infiltration.**

Reported By,

Dr. Mohamed Abdeltawab ,MSc

Dr. Mohamed Mohsen, MD

Ass. Prof. Mai Amr, Nuclear Medicine Consultant

30-Jul-2025 10:22 AM

PATIENT: AHMED HASSAN ABED HASSAN, 10125267832, Male, 47 y

Report version 3, 30-Jul-2025 10:22 AM

Page 2 of 2



www.Baheya.org



16602

مركز بيعة الهمد
4 شارع علوية أول شارع الهرم بجوار داري , الجيزة



مستشفى بهية الشيخ زايد
٤٠ أحي الأول - الشيخ زايد أمام كلية الهندسة



Patient Name	: الأستاذ / احمد حسن عابد حسن	Request No.	: 13009129
Patient No.	: 27709232100738	Request Date	: 31/07/2025
Path No.	: SR-1055/25	Verification Date	: 23/08/2025
Sex	: Male	Financial Category	: جوستاف روسي - نفقة الدولة
Age	: 47 Y	Department	: Out Patient
Referring Doc	:	Registered by	: marwa mohamed youss

Pathology Report

Clinical data

Right axillary lymphadenopathy .

Nature of specimen

Block & slide revision .

Gross specimen

One block and 8 slides labelled by (3307-25) were received .

Technique:

- Sections were prepared from the paraffin block, processed in Bench Mark ULTRA machine (Ventana) and stained by rabbit monoclonal antibodies using ultra view HRP DAB as chromogen and hematoxylin as counter stain. The sections treated against BCL6-clone(G/191E/A8), OCT2.CD15 (MMA)CD15 clone(MMA)CD4 clone (SP35).

Microscopic examination

Examination revealed effacement of nodular architecture by vague nodules formed of large atypical cells with background of mixed inflammatory infiltrate .

IPT :

CD20:Positive in few collections

CD3:Positive in many small cells

CD5 :Positive in many small cells

CD4:Positive in many small cells .

KI67 :20 %

BCL2 :Few small collections of cells.

BCL6 :Present in individual large cells .

OCT2 :Present in individual large cells .

CD30 :Negative.

CD15 : Negative .

Verified by :

Dr.Heba Hasan



Patient Name	: الأستاذ / احمد حسن عابد حسن	Request No.	: 13009129
Patient No.	: 27709232100735	Request Date	: 31/07/2025
Path No.	: SR-1055/25	Verification Date	: 23/08/2025
Sex	: Male	Financial Category	: جوستاف روسي - نفقة الدولة
Age	: 47 Y	Department	: Out Patient
Referring Doc :		Registered by	: .marwa mohamed youss

Pathology Report

Diagnosis

Right axillary lymphadenopathy, slide&block revision :
Nodular lymphocytic predominant Hodgkin lymphoma.

Signature

Prof. Dr. Gamal Abdelatty
Dr.Menan Hosam .

Verified by :

Dr.Heba Hasan

Printing Date : 02/09/2025 17:10:09

Page 2 of 2

Printing By : .marwa mohamed youss



غرفة رقم (4) - الدور الأول - مبنى رقم (987) - كورنيش النيل - مصر القديمة - القاهرة - مصر
Room No. (4) - First Floor-Building No. 987- Nile Corniche - Misr El Qadema - Cairo, Egypt



Patient Name الاستاذ / احمد حسن عابد
Gender/Age Male - 47 Years
Referred By أ.د/ عمر رضا فتحي

Request Date : 07/07/2025 07:34:35 م

Reporting Date : 07/07/2025 11:02:29 م



Laboratory Report

Test	Result	Unit	Reference Range
Blood Glucose			
- Random Blood Sugar	81	mg/dl	60 - 160
Hepatitis Markers			
- HIV 1+2+O Ab.	Negative		Negative
- HBsAg	Negative		Negative
- HCV antibodies	Negative		Negative
LIVER FUNCTIONS			
- Serum GPT (ALT)	48	U/L	0 - 40
COAGULATION			
- Patient time	13.00	%	12 - 16
- Control time	13		13
- Patient Concentration	100		70 - 110
- INR	1		1 - 1.3
- Ratio	1		
KIDNEY FUNCTIONS			
- Creatinine (Serum)	0.93	mg/dL	0.5 - 1.3

Doctor Signature

جمهورية مصر العربية
بطاقة تحقيق الشخصية

احمد
حسن عابد حسن
مزغونه
مركز البدرشين - الجيزة

١٩٧٧/٠١ ٢٧٧.٠٩٢٣٢١.٠٠٧٣٨

LF1933147

٢٢
٢٥
٢٥



٢٠٢٥/٠١ ٢٧٧.٠٩٢٣٢١.٠٠٧٣٨

بدون عمل
متزوج مسلم ذكر

البطاقة سارية حتى ٢٠٢٢/٠٦/١٧

١

أحمد هارح
١٠٩٨١٢٠٧٨٠
الطبخ

فتح ملف محمد المرام ٢٠٢٥

٢٠١٩
٢٠٢٥

د. محمد عبد
٢٠٢٥/٩

٢٧

اسم المريض ثلاثي:
رقم الملف: ٥١٩/٢٥

Doctor's Orders اوامر الطبيب

التشخيص: NCLPHL نوع التغذية:

توقيع الطبيب	الأوامر	الوقت	التاريخ
4/9/2025	PT. CT. Stage III B. Cervical, SCEN Axillary, mediastinal, Abdominal & sple pathology :- NCLPHL CD20+ve, CD3+ve, CD5+ve ACE2, Ki67 20% CD30 & CD15 → -ve ECHO :- 5.7 virology :- HIV 1 & 2 -ve HBsAg -ve HCV AB For CHOP cycles then reassess full bbs		

جميع الأوامر الطبية لابد أن تكون واضحة وموقعة من الطبيب

PATIENT NAME: AHMED HASSAN ABED HASSAN**GENDER:** Male**DATE :** 29-Jul-2025 11:35 AM**PATIENT ID:** 10125267832**AGE:** 47 y**ORDER:** 3492571**Whole body [18F] FDG PET-CT Scan****CLINICAL HISTORY:**

-47 years old male patient, presented with recently diagnosed HL.
Reason for the study: detection of FDG avid lymphomatous lesions.

TECHNIQUE:

- **Radiopharmaceutical:** 18F FDG, intravenously.
- **Time from 18F-FDG injection to scan:** 60 min
- **PET/CT images** were acquired from the skull through the upper thighs & CT images were acquired for the purpose of anatomical localization and attenuation correction. Images were reviewed in the transaxial, coronal, and sagittal planes.
- **Patient's weight:** Kg - **Patient's height:** cm - **BI. Gluc. Level:** mg/dL
- **Additional notes:** SUV max normalized to body weight was used.
- **Reference hepatic activity (SUV max):** 2.7
- **Reference mediastinal blood pool activity (SUV max):** 2

FINDINGS:**Head and Neck:**

- **Multiple FDG avid right lower cervical and right supra-clavicular lymph nodes (levels III, IV and Vb) reaching 1.8x2.2 cm with SUVmax~13.8 at the latter group.**
- Normal diffuse brain activity with no hypermetabolic brain lesions.
- Physiologic FDG uptake is seen in the nasopharynx, oropharynx, salivary glands and larynx.
- Physiologic FDG uptake is seen in submandibular and parotid salivary glands.
- Normal appearance of thyroid gland with no FDG avid nodules detected.

Chest:

- **Multiple FDG avid right axillary lymph nodes (all levels), reaching 4.5x3.3 cm with SUVmax~14.6**
- **Multiple FDG avid mediastinal right para-tracheal, retro-caval, pre-vascular, posterior mediastinal, carinal and sub-carinal lymph nodes, the largest is seen at the sub-carinal group measuring 3.3x2.3 cm with**

PATIENT: AHMED HASSAN ABED HASSAN, 10125267832, Male, 47 y

Report version 3, 30-Jul-2025 10:22 AM

Page 1 of 2



SUVmax~14, while the most active is seen at retro-caval group measuring 2x1.8 cm with SUVmax~16.9.

- No FDG avid lung nodules or masses detected.
- Bilateral pulmonary scattered atelectatic bands.
- No metabolically active pericardial or pleural lesions detected.
- Physiologic FDG uptake is noted in the heart.

Abdomen and Pelvis:

- **Average sized spleen with increased heterogenous FDG uptake of SUVmax~7.2 at its lower pole, likely infiltrated.**
- **Few low grade FDG avid porta-hepatic lymph nodes reaching 1.7 cm with SUVmax~3.3**
- Physiologic diffuse FDG uptake is seen over average sized liver. No FDG avid focal lesions detected
- Physiologic diffuse FDG uptake is seen over average sized spleen with homogenous parenchymal texture and no FDG avid focal lesions seen.
- Normal appearance of both kidneys with no stones or back-pressure changes. Physiologic FDG excretion is seen in the kidneys and bladder.
- Normal appearance of the pancreas and both supra-renal with no FDG avid lesions detected.
- No evidence of ascites or abdominal collections noted.

Musculoskeletal:

- Diffusely activated bone marrow with low grade FDG uptake, likely reactive for lab correlation.
- Few small non FDG avid left femoral and pelvic bones sclerotic osseous lesions, likely bone islands.
- No focal active blastic or lytic bone lesions detected.

Conclusion:

FDG PET/CT study for a known case of HL revealed;

- **Multiple FDG avid supra-diaphragmatic lymphomatous nodal lesions.**
- **Few low grade metabolically active upper abdominal lymph nodes for close follow up.**
- **Average sized spleen with increased heterogenous FDG uptake as described, likely lymphomatous infiltration.**

Reported By,

Dr. Mohamed Abdeltawab ,MSc

Dr. Mohamed Mohsen, MD

Ass. Prof. Mai Amr, Nuclear Medicine Consultant

30-Jul-2025 10:22 AM

PATIENT: AHMED HASSAN ABED HASSAN, 10125267832, Male, 47 y

Report version 3, 30-Jul-2025 10:22 AM

Page 2 of 2



www.Baheya.org



16602

مركز بنية الهم
4 شارع علوية أول شارع الهرم بجوار داري , الجيزة



مستشفى بهية الشيخ زايد
٤٠ أحي الأول - الشيخ زايد أمام كلية الهندسة



Patient Name	: الأستاذ / احمد حسن عابد حسن	Request No.	: 13009129
Patient No.	: 27709232100738	Request Date	: 31/07/2025
Path No.	: SR-1055/25	Verification Date	: 23/08/2025
Sex	: Male	Financial Category	: جوستاف روسي - نفقة الدولة
Age	: 47 Y	Department	: Out Patient
Referring Doc	:	Registered by	: marwa mohamed youss

Pathology Report

Clinical data

Right axillary lymphadenopathy .

Nature of specimen

Block & slide revision .

Gross specimen

One block and 8 slides labelled by (3307-25) were received .

Technique:

- Sections were prepared from the paraffin block, processed in Bench Mark ULTRA machine (Ventana) and stained by rabbit monoclonal antibodies using ultra view HRP DAB as chromogen and hematoxylin as counter stain. The sections treated against BCL6 clone (G/191E/A8), OCT2.CD15 (MMA)CD15 clone (MMA)CD4 clone (SP35).

Microscopic examination

Examination revealed effacement of nodular architecture by vague nodules formed of large atypical cells with background of mixed inflammatory infiltrate .

IPT :

CD20: Positive in few collections

CD3: Positive in many small cells

CD5 : Positive in many small cells

CD4: Positive in many small cells .

KI67 : 20 %

BCL2 : Few small collections of cells.

BCL6 : Present in individual large cells .

OCT2 : Present in individual large cells .

CD30 : Negative.

CD15 : Negative .

Verified by :

Dr. Heba Hasan



Patient Name	: الأستاذ / احمد حسن عابد حسن	Request No.	: 13009129
Patient No.	: 27709232100735	Request Date	: 31/07/2025
Path No.	: SR-1055/25	Verification Date	: 23/08/2025
Sex	: Male	Financial Category	: جوستاف روسي - نفقة الدولة
Age	: 47 Y	Department	: Out Patient
Referring Doc :		Registered by	: .marwa mohamed youss

Pathology Report

Diagnosis

Right axillary lymphadenopathy, slide&block revision :
Nodular lymphocytic predominant Hodgkin lymphoma.

Signature

Prof. Dr. Gamal Abdelatty
Dr.Menan Hosam .

Verified by :

Dr.Heba Hasan

Printing Date : 02/09/2025 17:10:09

Page 2 of 2

Printing By : .marwa mohamed youss



غرفة رقم (4) - الدور الأول - مبنى رقم (987) - كورنيش النيل - مصر القديمة - القاهرة - مصر
Room No. (4) - First Floor-Building No. 987- Nile Corniche - Misr El Qadema - Cairo, Egypt



Patient Name الاستاذ / احمد حسن عابد
Gender/Age Male - 47 Years
Referred By أ.د/ عمر رضا فتاح

Request Date : 07/07/2025 07:34:35 م

Reporting Date : 07/07/2025 11:02:29 م



Laboratory Report

Test	Result	Unit	Reference Range
Blood Glucose			
- Random Blood Sugar	81	mg/dl	60 - 160
Hepatitis Markers			
- HIV 1+2+O Ab.	Negative		Negative
- HBsAg	Negative		Negative
- HCV antibodies	Negative		Negative
LIVER FUNCTIONS			
- Serum GPT (ALT)	48	U/L	0 - 40
COAGULATION			
- Patient time	13.00	%	12 - 16
- Control time	13		13
- Patient Concentration	100		70 - 110
- INR	1		1 - 1.3
- Ratio	1		
KIDNEY FUNCTIONS			
- Creatinine (Serum)	0.93	mg/dL	0.5 - 1.3

Doctor Signature